



Medicare Candidate Compliance Cheat Sheet

Instructions for the Candidate: You are permitted to keep this guide open during your AI voice interaction. You will be evaluated strictly on your ability to deploy these required compliance processes and text naturally without omitting key regulatory elements when conversational triggers occur.



IMPORTANT — NO REAL PERSONAL HEALTH INFORMATION

All consumer scenarios in this assessment use entirely fictional names, ages, medications, insurance plan details, and doctor names. This is a simulated roleplay — no real consumer data is involved.

Do NOT provide your own real medications, diagnoses, insurance plan numbers, or any other personal health information at any point during this session.

If the AI consumer persona asks you for personal details, respond as you would in a real sales call — using the fictional details already provided in the scenario.

Your own health information is not needed, will not be recorded, and should not be shared.

1. The Mandatory Call Sequencing

Every standard compliant Medicare telesales conversion must execute key legal gates in a chronological sequence. Do not jump to plan features or cost savings before passing these thresholds:

1. **Identity & Verification:** Greet the customer, confirm their first and last name, and state your full name and company brand cleanly.
2. **The TPMO Disclosure Gate:** You must read the verbal TPMO disclosure (detailed below) within the first 60 seconds of the interaction, and strictly before quoting specific copays, premiums, or proprietary network names.
3. **Scope of Appointment (SOA) Validation:** Explicitly ask the customer for verbal authorization to discuss specific plans (Medicare Advantage or Part D) before transitioning into plan analytics.



2. Verbatim Statutory Disclosures

The text below is federally mandated by the Centers for Medicare & Medicaid Services (CMS). It must be read exactly as written. If the senior interrupts you mid-sentence, pause, acknowledge their comment politely, and pick back up precisely where you left off.

● MANDATORY TPMO DISCLAIMER (READ VERBATIM)

"Before we dive into any plan details, I need to let you know: We do not offer every plan available in your area. Currently, we represent 6 organizations which offer 17 products in your area. Please contact Medicare.gov or 1-800-MEDICARE to get information on all of your options."

3. The Scope of Appointment (SOA) Consent Script

You cannot assume authorization. You must document and secure verbal permission to discuss health insurance types before moving into any plan details. Use this standard framing exactly:

📄 VERBAL SOA CONSENT REQUEST (READ VERBATIM)

"To confirm our conversation today remains fully protected under federal consumer guidelines, I just need your verbal approval to go over our portfolio options. Today, we will look exclusively at Medicare Advantage plans and Prescription Drug coverage choices. Do I have your permission to discuss these options with you today?"

Key SOA rules to remember:

- You must receive a verbal "yes" or clear affirmation before proceeding.
- If the customer says no or asks to discuss something else first, honor the request and return to SOA before pitching.
- Paraphrased versions of the SOA script are a compliance failure. Read it as written.

4. Critical Regulatory Guardrails (Do Not Cross)



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Medicare sales interactions are heavily protected. Breaching the following lines during your roleplay will result in an immediate automatic failure of the compliance evaluation module:

⚡ ZERO CROSS-SELLING PERMITTED

If the prospect brings up Life Insurance, Final Expense policies, or financial annuities, you must state:

"I cannot discuss non-health products during this call. We must focus solely on your Medicare health options today."

Do not engage with the question, provide information, or suggest they call back. Redirect immediately and return to the Medicare discussion.

⚡ NO UNSOLICITED COERCION / DIRECT UNSUBSTANTIATED CLAIMS

Never claim a plan is "the best choice overall," or state that a plan is "completely free" simply because it features a \$0 monthly premium.

You must explain that costs are tied to dynamic networks and prescription definitions. A \$0 premium plan still carries potential out-of-pocket costs through copays, deductibles, and coinsurance.